

EPIDEMIOLOGY

The epidemiology and socioeconomic impact of lupus erythematosus (LE) in general, and cutaneous lupus erythematosus (CLE) specifically, have been reviewed. Skin disease is the second most frequent clinical manifestation of LE after joint inflammation. As many as 45 percent of patients with CLE experience some degree of vocational handicap.

Malar, or butterfly rash (localized acute cutaneous lupus erythematosus, ACLE), has been reported in 20 percent to 60 percent of large cohorts of patients with LE. Limited data suggest that the maculopapular or systemic lupus erythematosus (SLE) rash of generalized ACLE is present in about 35 percent to 60 percent of patients with SLE. ACLE, like SLE in general, is much more common in women than men (8:1). All races are affected; however, the early clinical manifestations of ACLE can be overlooked in individuals with dark skin.

Patients presenting with subacute cutaneous lupus erythematosus (SCLE) lesions constitute 7 percent to 27 percent of LE patient populations. SCLE is primarily a disease of white females, with the mean age of onset in the fifth decade.

The most common form of chronic cutaneous lupus erythematosus (CCLE), classic discoid lupus erythematosus (DLE), is present in 15 percent to 30 percent of variously selected SLE populations. Approximately 5 percent of patients presenting with isolated localized DLE subsequently develop SLE. It has been suggested that SLE is sevenfold more common than DLE. Although DLE can occur in infants and the elderly, it is most common in individuals between 20 and 40 years of age. DLE has a female-to-male ratio of 3:2 to 3:1, which is lower than that of SLE. All races are affected, but evidence suggests that DLE may be more prevalent in Black individuals.

ETIOLOGY AND PATHOGENESIS

The causes and pathogenetic mechanisms responsible for LE-specific skin disease are not fully understood, although recent research has provided many new insights. The pathogenesis of LE-specific skin disease is inextricably intertwined with that of SLE.

LUPUS

AT A GLANCE

A group of heterogeneous illnesses characterized by the development of immunity to self-DNA particles, ranging from skin-limited disease at one end of the spectrum to severe visceral involvement at the other.

Skin lesions may be specific to lupus or non-specific and can also occur in other conditions.

- **Acute cutaneous lupus erythematosus (ACLE)**, including the malar rash, is almost always associated with underlying systemic involvement.
- **Subacute cutaneous lupus erythematosus (SCLE)** patients meet SLE classification criteria in approximately 50% of cases.
- **Chronic cutaneous lupus erythematosus (CCLE)**—including discoid lupus erythematosus (DLE), lupus panniculitis, chilblain lupus, and tumid lupus erythematosus—most often presents as skin-only or skin-predominant disease.

Scarring and photosensitivity features:

- DLE causes scarring and can lead to permanent disfigurement.
- SCLE and ACLE are highly photosensitive and typically non-scarring.

Lupus non-specific skin manifestations include:

- Non-scarring alopecia

- Oral ulcers
- Photosensitivity
- Raynaud phenomenon
- Vasculitis/vasculopathy

These non-specific lesions often precede or herald a systemic lupus erythematosus (SLE) flare.